

BRN Case Management CE Package - Draft / Pending BRN CEP Approval

Care Coordination Across Disciplines and Settings

Provider (from master packet): CI Institute of Nursing, Inc. — 419 E. Hamilton Ave., Campbell, CA 95008 — 650-449-6706 — humanresources@ci-ion.com — <https://www.ci-ion.com>

"Pending California Board of Registered Nursing Continuing Education Provider approval. Do not advertise, issue certificates, or represent this course as approved until a BRN CEP number has been issued."

This document follows a CCCCCO-style documentation organization only (structured overview, objectives, content outline, activities, facilitator notes, assessment mapping, compliance support). No CCCCCO content is copied; all content is original and customized for California BRN Case Management CE.

Module At a Glance

Field	Value
Module ID	CM-M03
Module title	Care Coordination Across Disciplines and Settings
Parent course	RN Case Management in Home Health: Care Coordination, Documentation, Patient Advocacy, and Safe Transitions of Care
Contact hours	3 (draft design math)
Estimated learner minutes	165 (>=150 floor)
Direct/indirect linkage	Direct + indirect patient/client care
Approval status	Draft / Pending BRN CEP Approval

BRN Relevance Rationale

Nurse-led interdisciplinary coordination protects continuity and prevents gaps that cause patient harm.

Module Description

Develops the RN's coordinating role across the interdisciplinary team and care settings using structured handoff communication and loop-closure tracking.

Measurable Learning Objectives

By the end of this module the learner will be able to:

- 1 Map an interdisciplinary team and each member's contribution to a care plan.
- 2 Apply structured handoff communication (e.g., SBAR) to a coordination event.
- 3 Identify two failure points in cross-setting transitions and a nursing mitigation for each.
- 4 Distinguish information that may be shared under minimum-necessary principles.

Required Readings / Learning Content Sections

- Reading 3.1 - The interdisciplinary team and the RN's coordinating role.
- Reading 3.2 - Structured communication and SBAR.
- Reading 3.3 - Cross-setting coordination (home health, primary care, specialty, payer).
- Reading 3.4 - Closing the loop and follow-up tracking.

Detailed Lesson Outline

Lesson ID	Lesson	Est. minutes
CM-M03-L1	The interdisciplinary team & RN's coordinating role	35
CM-M03-L2	Structured communication / SBAR	40
CM-M03-L3	Cross-setting coordination	35
CM-M03-L4	Closing the loop & follow-up tracking	35
CM-M03-KC	Knowledge check + coordination map	20

Total estimated instructional minutes: **165** (meets the ≥ 150 -minute / 3-contact-hour floor).

Key Terms

Term	Definition
SBAR	Situation-Background-Assessment-Recommendation: a structured handoff format.
Interdisciplinary team	Clinicians and staff from multiple disciplines collaborating on care.
Loop closure	Confirming that a referral/order/result was completed and acted upon.
Minimum necessary	Sharing only the information needed for the purpose.
Transition point	A handoff between settings/providers where care can break down.

Fictional / De-identified Case Scenario

All clinical content is fictional and de-identified. No PHI. A fictional client is discharged from a hospital to home health, but the medication reconciliation is missing and the primary care follow-up is unscheduled.

Learner Activities

- Read Sections 3.1-3.4.
- Build an interdisciplinary team map for the scenario.
- Write a 3-line SBAR to the primary care office.
- Complete the branching coordination scenario and Knowledge Check 3.

Documentation / Decision-Making Exercise

Build an Interdisciplinary Coordination Map (who / what / when / loop-closure) for the scenario and write a 3-line SBAR handoff.

Knowledge Check Blueprint

8 items + 1 branching-lesson decision path, unlimited attempts, immediate feedback, no full key, $\geq 80\%$ to complete.

Assessment Items (Learner-Facing Stems)

KC3-Q1 (MCQ). Which tool best structures a nurse-to-provider handoff?

- SOAP
- SBAR
- DSA
- SWOT

KC3-Q2 (MCQ). The missing medication reconciliation in the scenario is best described as a:

- bullet All loop-closure failure at a transition point
- bullet Billing dispute
- bullet Normal finding
- bullet Scope violation

KC3-Q3 (True/False). Under minimum-necessary principles, the RN should share only the information needed for the coordination purpose.

- bullet True
- bullet False

LMS Completion Criteria

- bullet All lesson pages viewed (activity completion: require view).
- bullet M4-M03 knowledge check passed at >=80% (require passing grade).
- bullet Documentation/decision exercise submitted (require submission).

Evidence Generated by Learner Completion

- bullet Activity-completion timestamps (time-on-task evidence).
- bullet Knowledge-check score and attempt log.
- bullet Submitted exercise artifact (retained >=4 years; no PHI).

Moodle Activity Recommendations

- bullet Lesson (branching)
- bullet Web
- bullet Database
- bullet Quiz (KC3)
- bullet Assignment (map)

Accessibility Considerations

- bullet Captions + transcripts for any media; semantic headings; alt text on diagrams.
- bullet Sufficient color contrast; no color-only meaning; keyboard-navigable activities.
- bullet Accessible downloadable templates (tagged PDF + DOCX); extended-time accommodation supported.
- bullet Target WCAG 2.1 AA on the built course.

PHI / Privacy Guardrails

- bullet All cases fictional and de-identified; no real patient/resident/family/facility data.
- bullet All team/member/facility names must be fictional.
- bullet All communication reminders and (for capstone) attestation + manual PHI review.

SME Review Flags

[[SME REVIEW REQUIRED]] Confirm SBAR/handoff examples reflect current practice.

Compliance Review Flags

[[COMPLIANCE REVIEW]] All team/member/facility names must be fictional.

Internal Answer Key Appendix (ADMIN / FACULTY ONLY - DO NOT SHOW LEARNERS)

Item ID	Type	Correct answer	Rationale	Remediation
KC3-Q1	MCQ	B	SBAR is a structured handoff communication tool.	Review Reading 3.2 (SBAR).
KC3-Q2	MCQ	A	A required step did not close the loop at the hospital-to-home transition.	Review Reading 3.3/3.4.
KC3-Q3	True/False	True	Minimum-necessary limits disclosure to what the purpose requires.	Review Reading 3.4.

Debrief / Remediation Guidance

Highlight how a single unclosed loop (med rec) cascades into harm, and how SBAR plus tracking prevents it. Failed attempts route to Reading 3.2-3.4.

Application-Packet Defensibility Notes

Coordination and handoff communication directly protect continuity of patient care - clearly nursing-relevant indirect/direct care.

Program-structure note: This module is part of the 30-contact-hour, 10-module RN case management program (10 modules x 3 contact hours; 50 instructional minutes = 1 contact hour; minimum 1,500 instructional minutes, 1,680 design minutes). The official BRN contact-hour map and transfer worksheet are aligned to this 30-contact-hour structure. Any earlier 4-contact-hour representative course is superseded and is not part of the current BRN CEP signer-review package. [[SME REVIEW REQUIRED BEFORE SUBMISSION]]

Do not advertise, issue certificates, or represent this course as BRN-approved until the official BRN CEP approval and provider number are issued.